

Hair Transplantation

The success of hair transplantation depends on the availability of sufficient permanent donor hair and the absence of overlying diffuse telogen effluvium (Table 8).

Final results can be assessed at 9–12 months post-procedure. In many cases, multiple surgical sessions are necessary to achieve optimal outcomes.

The best long-term results are observed in patients with medically controlled or spontaneously stabilized androgenetic alopecia (AGA). In men, combining finasteride 1 mg and/or topical minoxidil with follicular unit transplantation may help reduce postoperative progression of AGA.

Body dysmorphic disorder or unrealistic patient expectations are contraindications for this aesthetic procedure.

Platelet-Rich Plasma (PRP)

Efficacy – Males

Two studies evaluating PRP efficacy in male and female patients with AGA met the inclusion criteria for the guideline, both providing grade C evidence, resulting in a level of evidence 3.^{105,106}

Efficacy – Females

Three studies assessing PRP in female patients were included in the evidence-based evaluation.^{105–107} Two of these studies included both male and female participants, with one study enrolling only two women.^{105,106} The levels of evidence were graded as B, C, and C, leading to an overall level of evidence 3.

Increases in hair density were reported across the studies. However, limitations include small sample sizes, heterogeneous PRP protocols, lack of control groups,

and pooled reporting of results for men and women in two of the three studies. Except for one study, long-term follow-up data are lacking.

Based on current evidence, there is **limited support** for the use of PRP in treating AGA in both men and women (Table 9).

Instructions for Use / Practicability / Side Effects

There is **no standardized protocol** for PRP treatment in AGA. Techniques for platelet isolation and activation, as well as dosage, injection frequency, and treatment intervals, vary significantly across studies and clinical practices. This lack of standardization limits comparability and reproducibility.

¹⁰⁵⁻¹⁰⁷ (References removed per instruction)